

INDICATIONS FOR TESTING

Offer one-time screening to all individuals 13-64 yrs of age, routine prenatal screening to pregnant individuals, and more frequent screening to individuals with risk factors^a

Point-of-Care Rapid Screening^b

ORDER
Rapid HIV-1/2
point-of-care test

Nonreactive
for HIV-1/2

HIV not
detected^d

Reactive^d for
HIV-1/2

Preferred Laboratory Screening^c

ORDER
Fourth generation HIV-1/2 Ag/Ab
combination immunoassay

Nonreactive
for HIV-1/2^e

Reactive for
HIV-1/2

ORDER
HIV-1/2 Ab differentiation immunoassay
(testing with reflex to NAAT recommended)

HIV-1 Ab nonreactive or indeterminate
AND
HIV-2 Ab nonreactive or indeterminate

HIV-1 Ab reactive
AND
HIV-2 Ab reactive

HIV-1 Ab reactive
AND
HIV-2 Ab indeterminate

HIV-1 Ab reactive
AND
HIV-2 Ab nonreactive

HIV-1 Ab nonreactive
AND
HIV-2 Ab reactive

ORDER (OR REFLEX TO)
HIV by NAAT

Untyped HIV
infection

HIV-1
infection^f

HIV-1
infection

HIV-2
infection

HIV-1
negative

HIV-1
positive

HIV-1 not
detected^g

Acute HIV-1
infection

Abbreviations

Ab	Antibody
Ag	Antigen
CLIA	Clinical Laboratory Improvement Amendments
NAAT	Nucleic acid amplification testing

^aMen who have sex with men, and individuals with HIV-positive sexual partners, multiple sexual partners, or history of blood transfusion, intravenous drug use, or another sexually transmitted infection.

^bA fourth generation immunoassay is preferred as a diagnostic starting point. However, when preferred testing is unavailable, a rapid assay with appropriate follow-up testing may be considered.

^cFollowing a preliminary positive result from any CLIA-waived setting, laboratories should begin testing with an Ag/Ab immunoassay using plasma or serum.

^dWhen using a rapid Alere Determine assay, follow up a reactive result with an HIV-1/2 Ab differentiation assay; any other rapid assay should be followed by a fourth generation immunoassay.

^eConsider patient history to inform the need for further testing.

^fNo additional testing is required unless HIV-2 infection is strongly suspected.

^gConsider patient history to determine if additional testing, such as NAAT for HIV-2 (if not already performed), is warranted.

Reference

U.S. Preventive Services Task Force, Owens DK, Davidson KW, et al. [Screening for HIV infection: U.S. Preventive Services Task Force recommendation statement](#). JAMA. 2019;321(23):2326-2336.